

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

September 6, 2023

MEMORANDUM FOR:

(b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE
Assistant Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

THROUGH:

(b)(6),(b)(7)(C),(b)(7)(F) MD
Deputy Assistant Director of Clinical Services/Medical Director
ICE Health Service Corps

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Chief of Staff
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FROM:

CAPT (b)(6),(b)(7)(C),(b)(7)(F) PA-C
Chief, Investigation Unit
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

SUBJECT:

Report of Findings – Mortality Review
Nebane ABIENWI, (b)(6),(b)(7)(C)

Executive Summary:

On October 1, 2019, U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of ICE noncitizen Nebane ABIENWI, (b)(6),(b)(7)(C). During his detention period, ICE housed Mr. ABIENWI, a 37-year-old Cameroon male, at Otay Mesa Detention Center (OMDC) in San Diego, California (CA), from September 19 until October 1, 2019, the date of his death. Mr. ABIENWI's death certificate showed his cause of death as hypertensive basal ganglia hemorrhage.

On August 30, 2019, prior to Mr. ABIENWI's ICE custody, Centro De Salud Tijuana provided him medical care for essential hypertension and prescribed captopril (an antihypertensive medication) and ranitidine (an antacid medication).

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On September 5, 2019, Mr. ABIENWI presented to the San Ysidro Port of Entry in San Ysidro, CA, via the pedestrian primary inspection lane, claiming credible fear, and requesting political asylum. U.S. Customs and Border Protection (CBP) officers detained Mr. ABIENWI at the San Ysidro Admissibility Enforcement Unit in San Ysidro, CA.

On September 11, 2019, while in CBP custody, a “Mission Medical Support” advanced practice provider (APP) evaluated Mr. ABIENWI for constipation, rash, and prescribed him medications. The APP did not document Mr. ABIENWI’s history of hypertension (HTN), but documented normal vital signs (VS), a normal physical examination, except for a skin rash, and cleared him for transport and incarceration.

On September 18, 2019, CBP transferred Mr. ABIENWI to ICE custody in “good health,” and housed him at Otay Mesa Detention Center in San Diego, CA. A registered nurse (RN) completed Mr. ABIENWI’s pre-screen evaluation and documented “patient speaks English fluently.” Mr. ABIENWI confirmed a history of past/current illness and taking medications. The RN assigned Mr. ABIENWI an expedited intake screening.

On September 19, 2019, an RN completed Mr. ABIENWI’s intake screening; however, a radiology technician completed Mr. ABIENWI’s vital signs (VS) and documented the results on the back of Mr. ABIENWI’s medical consent form for an RN to enter the results into the electronic health record (eHR) at a later time. Mr. ABIENWI reported the following history: a three-day hospitalization for elevated blood pressure readings (diagnosed with HTN on August 23, 2019), taking antihypertensive medications for two weeks, but discontinued by a doctor because his “BP was already normal,” sexual abuse (December 2017), and drinking four cans of beer, rarely (last consumed on September 4, 2019). The RN documented normal VS, a normal intake screening, except for lower abdominal rash, and auto-created a routine physical examination in ten days for a simple physical exam (PE-S). Mr. ABIENWI did not receive a higher-level provider evaluation to further review his medical conditions or medications.

On September 26, 2019, at approximately 3:24 a.m., OMDC’s custody officer called a medical emergency for Mr. ABIENWI, after he fell off his top bunk. An RN responded to the medical emergency and observed Mr. ABIENWI confused, sweating, not responding to commands, unable to move left side extremities, having “jerky movements,” The RN documented normal VS, except for an elevated blood pressure reading of 180/100 millimeters of mercury [(mmHg; normal range is more than 90/60 and less than 120/80 mmHg]. OMDC called emergency medical services (EMS). Upon arrival to OMDC, EMS personnel assumed care of Mr. ABIENWI and transported him to Sharp Chula Vista Medical center (SCVMC) in Chula Vista, CA.

While at SCVMC, an emergency department physician (MD) completed Mr. ABIENWI’s physical examination, documented his inability to follow commands, his nonverbal responses, and inability to open his eyes. The MD intubated (insertion of a tube into the trachea for ventilation) Mr. ABIENWI, initiated ventilator-assisted breathing, and consulted a neurosurgeon,

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who admitted him to the intensive care unit (ICU), with a diagnosis of acute severe right intraparenchymal hemorrhage (bleeding within the brain), documented a poor prognosis, and not a candidate for surgical intervention.

On October 1, 2019, at approximately 12:05 p.m., two SCVMC MD's confirmed Mr. ABIENWI's clinical examination was consistent with "brain death" and pronounced him dead.

- At approximately 12:38p.m., Mr. ABIENWI's wife was notified of Mr. ABIENWI's death.
- At approximately 2:37 p.m., SCVMC discontinued Mr. ABIENWI's ventilator support.

Mortality Finding:

As a result of Mr. ABIENWI's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures, or practices are warranted; and identify issues that require further study.

A mortality review determined the care OMDC provided Mr. ABIENWI deviated beyond safe limits but did not directly contribute to his death. Specifically, the mortality review participant determined the following: 1) lack of review/acknowledgment of Mr. ABIENWI's medical records, prior to ICE custody for continuity of care purposes; 2) missing or lack of documentation, which included his outgoing referral; and 3) untimely referral to a higher-level provider for further evaluation of his underlying medical conditions.

On August 25, 2023, Fact Finder (FF) CAPT (b)(6),(b)(7)(C),(b)(7)(F) discussed the events surrounding Mr. ABIENWI's death with Deputy Assistant Director of Clinical Services/Medical Director, Dr. (b)(6),(b)(7)(C),(b)(7)(F) physician reviewer.

The following summary of events was based on: 1) Mr. ABIENWI's OMDC medical records; 2) Mr. ABIENWI's incident and hospital updates; 3) incident and notification reports; 4) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 5) Mr. ABIENWI's detention file; 6) video footage from September 26, 2020; 7) Mr. ABIENWI's death certificate; 8) an on-site review and staff interviews conducted by FF CDR (b)(6),(b)(7)(C),(b)(7)(F) on November 4 to 7, 2019 and 9) applicable OMDC and IHSC policies and procedures, and ICE Detention Standards.

Summary of Events

On September 18, 2019, at approximately 9:00 p.m., an RN completed Mr. ABIENWI's pre-screen evaluation and documented "patient speaks English fluently." Mr. ABIENWI confirmed a

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history of past/current illness and taking medications. The RN assigned Mr. ABIENWI for an expedited intake screening.

On September 19, 2019, an RN completed Mr. ABIENWI's intake screening; however, a radiology technician completed Mr. ABIENWI's VS and documented the results on the back of the medical consent form for the RN to review and enter his eHR at a later time. Mr. ABIENWI reported the following history: a three-day hospitalization for elevated blood pressure readings (diagnosed with HTN on August 23, 2019), taking antihypertensive medications for two weeks, but discontinued by a doctor because his "BP was already normal," sexual abuse (December 2017), and drinking four cans of beer, rarely (last consumed on September 4, 2019). The RN documented normal VS, a normal intake screening, except for lower abdominal rash, and auto-created a PE-S appointment in ten days. Mr. ABIENWI did not receive a higher-level provider evaluation to further review his past medical history or medications.

Of note, Mr. ABIENWI's OMDC medical records included a medical certificate for care provided prior to federal custody, dated August 30, 2019, from "Centro De Salud Tijuana" that outlined Mr. ABIENWI's past medical history of essential HTN, medication orders for captopril 25 mg, one tablet, twice a day for thirty days, and ranitidine 150 mg, one tablet, twice a day for thirty days.

On September 26, 2019, at approximately 3:24 a.m., OMDC's custody officer called a medical emergency for Mr. ABIENWI, after he fell off his top bunk, landed on blankets on a chair, and sat on the floor. An RN responded to the medical emergency and observed Mr. ABIENWI confused, sweating, not responding to commands, unable to move left side extremities, having "jerky movements," in all extremities, and complaining of thirst. Approximately fifteen minutes later, the RN documented Mr. ABIENWI's ability to speak and respond to questions; however, he was only alert and oriented to his name. The RN documented relatively normal VS, except for an elevated blood pressure reading of 180/100 mmHg. OMDC custody staff called 911. Upon arrival, emergency medical services (EMS) personnel assumed care of Mr. ABIENWI and transported him to SCVMC in Chula Vista, CA.

- At SCVMC, an emergency department MD completed Mr. ABIENWI's physical examination, documented his inability to follow commands, his nonverbal responses, and inability to open his eyes. The MD intubated Mr. ABIENWI, initiated ventilator-assisted breathing, and consulted a neurosurgeon, who admitted him to the ICU, with a diagnosis of acute severe right intraparenchymal hemorrhage (bleeding within the brain). The MD documented a poor prognosis, and noted Mr. ABIENWI was not a candidate for surgical intervention.

On September 27, 2019, because of Mr. ABIENWI's poor prognosis, SCVMC's medical team issued a do-not-resuscitate (DNR) order, due to the lack of available surrogate decision makers (also known as health care proxy or agents who make medical decisions for a patient who is unable to make decisions for themselves).

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From September 28 to 30, 2019, Mr. ABIENWI remained in the ICU, with multi-system organ involvement, declining neurological examinations, and a high risk for morbidity and mortality.

On October 1, 2019, at approximately 12:05 p.m., Two SCVMC MD's confirmed Mr. ABIENWI's clinical examination consistent with "brain death" and pronounced him dead.

- At approximately 12:38p.m., Mr. ABIENWI's wife was notified of Mr. ABIENWI's death.
- At approximately 2:37 p.m., SCVMC discontinued Mr. ABIENWI's ventilator support.

The final autopsy report identified Mr. ABIENWI's cause of death as hypertensive basal ganglia hemorrhage.

ICE detention standards used for this review: Performance-Based National Detention Standards (PBNDS), 2011 with 2016 Revisions. The following are identified program strengths, weaknesses, and recommendations found during this review.

Strengths

Strengths identified during this investigation were: 1) on September 26, 2019, OMDC's medical team responded to Mr. ABIENWI's medical emergency call timely; 2) OMDC nursing staff completed Mr. ABIENWI's pre-screen and intake screening within the allotted 12-hour time frame; and 3) OMDC medical staff provided requested information timely and remained receptive to feedback.

Weaknesses

1. Patient assessments and documentation.

- OMDC medical staff did not perform a confirmatory sign-off of Mr. ABIENWI's medical records reflecting care received prior to his ICE custody, acknowledging receipt, review, or forwarding of the document to a higher-level provider to review for continuity of care purposes.
- On September 19, 2019, a radiology technician completed Mr. ABIENWI's VS; however, the technician documented the information on the back of Mr. ABIENWI's medical consent form and did not enter the information directly into the electronic medical record. Review of the radiology technician position description did not list completion of VS as part of a radiology technician's scope of practice.
- On an unknown date, an unknown author documented chronic heart burn on the back of Mr. ABIENWI's medical consent form; however, Mr. ABIENWI's electronic medical record did not include this information within the intake note.
- On September 19, 2019, an RN documented a normal intake; however, Mr. ABIENWI reported a history of drinking beer, past hospitalization for high blood pressure in August

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2019, taking medications for two weeks, and using a cream for a rash. The RN auto-created a PE-S, instead of a complex PE-C, which would have indicated the need for a higher-level provider evaluation.

- Between September 26 and October 1, 2019, Mr. ABIENWI's medical record did not include daily hospital updates during his admission at SCVMC.
- On September 26, 2019, the day of Mr. ABIENWI's transport to SCVMC, OMDC medical staff did not complete an outgoing referral, per IHSC policy.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section BB. *Medical Records*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section M. *Comprehensive Health Assessment*.
- NCCHC 2018 standards: J-A-08, *Health Records*.
- NCCHC 2018 standards: J-E-09, *Continuity, Coordination, and Quality of Care During Incarceration*.

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director of Health Care Compliance (DAD of HCC) for awareness.
- Since this mortality occurred on September 26, 2019, the IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, OMDC administrator and health authority for awareness and to implement any action (s) deemed appropriate.

End of report.